

Public Health Emergency Operations Center

"COUSP DRC"

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°070/MVB_23/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (48)	• Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba • Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Masereka and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (3/23): Lubunga, Makiso-Kisangani and Mangobo
Reporting date	July 23, 2026
Publication date	July 24, 2026

INDICATORS KEYS — CURRENT TO JULY 23, 2026

CONFIRMED CASES — 5 PROVINCES 2,973 +68 new cases (Ituri 51 · N-Kivu 15- Haut-Uele 2) 	CONFIRMED DEATHS · LEATHITIS 1,309 · (44.0%) +40 deaths (Ituri 30 · North Kivu 10) 	SUSPECTED CASES OF THE DAY 315 Ituri 199 · N-Kivu 107 · Tshopo 9 
PATIENTS IN ISOLATION 766 Ituri 570 · N-Kivu 192 – Tshopo 4 	CURED — CUMULATIVE 540 +21 recovered (Ituri 20 – North Kivu 1) 	NATIONAL CONTACT TRACKING 73.9% 11,351/ 15,358 views · Ituri 72.1% - N-Kivu 78.9% - Tshopo 100.0% 

1. Epidemiological Highlights

- **Epidemiological situation:** A new health zone has been affected in the Haut-Uélé province: the Rungu health zone. The total number of affected health zones now stands at 48 out of 140 across the five provinces, of which 47 remain in active transmission.
- **Coordination:** Official visit of Her Excellency the Prime Minister to the provinces of Tshopo and Ituri to strengthen activities to combat Ebola virus disease (EVD); Installation and launch of diagnostic laboratory activities in Kasenyi in the Tchomia health zone in Ituri.
- **Case trends:** In the last 24 hours, 68 new confirmed cases, 40 confirmed deaths and 21 recoveries were recorded:
 - o **Ituri:** 51 new confirmed cases were notified, mainly in Bunia (17), Rwampara (9), Nizi (8), Mangala (6), Komanda (3), Lita (2), Mongbwalu (2), Damas (1), Logo (1), Rimba (1) and Tchomia (1), 30 deaths among which 60% were community deaths.
 - o **North Kivu:** 15 new confirmed cases were reported (Katwa 12, Butembo 1, Musienene 1, and Beni 1) as well as 10 deaths, 80% of which were community deaths and 2 at treatment centers. The provincial total reaches 290 confirmed cases and 182 deaths, representing a case fatality rate of 62.8%.

In **Haut-Uélé**, two new confirmed cases (one in Rungu and one in Wamba) and no deaths were reported. A new health zone has been affected: the **Rungu health zone**. The provincial total now stands at 29 confirmed cases and 15 deaths, representing a case fatality rate of 51.7%, spread across 5 of the province's 13 health zones.

o **South Kivu and Tshopo** : No new confirmed cases or deaths have been reported. The totals remain at 5 cases, 4 deaths, and 1 recovery in Tshopo, and 3 cases, including 1 death, in South Kivu, where the Miti-Murhesa health zone has a total of 56 days without a new confirmed case.

- **Analysis:** Ituri remains the epicenter of the epidemic, accounting for 89.0% of cumulative cases and 75.0% of new daily confirmations. Contact tracing performance remains insufficient and requires strengthened surveillance activities.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 48 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.

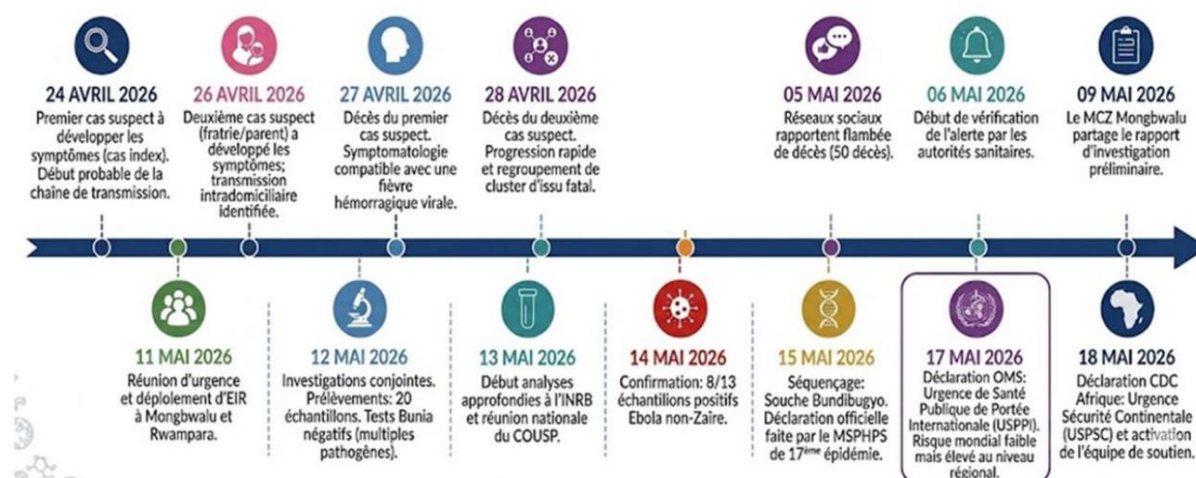


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	2,646	1107	41.8%	28 / 36 (77.8%)	51
North Kivu	290	182	62.8%	11/34 (32.4%)	15
Haut-Uélé	29	15	51.7%	5/13 (38.5%)	2
Tshopo	5	4	80.0%	3/23 (13.0%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	2,973	1,309	44.0%	48 out of 140	68

Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 89.0% of confirmed cases (n = 2,646) and 84.6% of deaths (n = 1,107), representing a case fatality rate of 41.8%. The health zones of Bunia (713 cases), Rwampara (490), Mongbwalu (484 cases), Nizi (316), Nyankunde (114), and Lita (103 cases) remain the most affected, accounting for approximately 83.9% of reported cases at the provincial level and 74.7% at the national level.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	2,646	1107	41.8%	51	18	12	30
Adja	10	1	10.0%				
Aru	5	1	20.0%				
Ariwara	7	1	14.3%				
Aungba	4	4	100.0%				
Bamboo	51	10	19.6%	0	0	1	1
Boga	1	1	100.0%				
Bunia	713	185	25.9%	17	6	4	10
Damascus	14	8	57.1%	1	0	0	0
Drodro	6	4	66.7%				
Fataki	17	6	35.3%				
Gethy	1	0	0.0%				
Kambala	2	0	0.0%				
Kilo	23	11	47.8%				
Komanda	33	22	66.7%	3	1	0	1
Lita	103	59	57.3%	2	1	0	1
Logo	9	3	33.3%	1	0	0	0
Lolwa	4	1	25.0%				
Mahagi	1	1	100.0%				
Mambasa	7	4	57.1%				
Mandima	13	9	69.2%				
Mangala	92	60	65.2%	6	1	0	1
Mongbwalu	484	228	47.1%	2	1	1	2
Nia Nia	78	43	55.1%	0	0	2	2
Nizi	316	151	47.8%	8	5	2	7
Nyankunde	114	30	26.3%				
Rimba	8	4	50.0%	1	0	0	0
Rwampara	490	236	48.2%	9	3	2	5
Tchomia	40	24	60.0%	1	0	0	0
North Kivu	290	182	62.8%	15	8	2	10
Blessed	42	29	69.0%	1	0	0	0
Butembo	63	41	65.1%	1	1	1	2
Goma	1	0	0.0%				
Kalunguta	4	3	75.0%				
Katwa	133	85	63.9%	12	7	1	8
Kyondo	10	7	70.0%				

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Mabalako	1	0	0.0%				
Masereka	4	0	0.0%				
Musienene	27	13	48.1%	1	0	0	0
Oicha	4	3	75.0%				
Vuhovi	1	1	100.0%				
South Kivu	3	1	33.3%	—			—
Miti-Murhesa	3	1	33.3%				
Haut-Uélé	29	15	51.7%	2	0	0	0
Boma Mangbetu	6	2	33.3%				
Isiro	8	5	62.5%				
Pawa	7	5	71.4%				
Rungu	1	0	0.0%	1			
Wamba	7	3	42.9%	1			
Tshopo	5	4	80.0%	—			—
Lubunga	1	0	0.0%				
Makiso-Kisangani	3	3	100.0%				
Mangobo	1	1	100.0%				
Total	2,973	1,309	44.0%	68	26	14	40

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi, Nyankunde and Lita, with continued transmission in the areas already affected and an extension towards the new health zone of Rungu in Haut-Uélé.

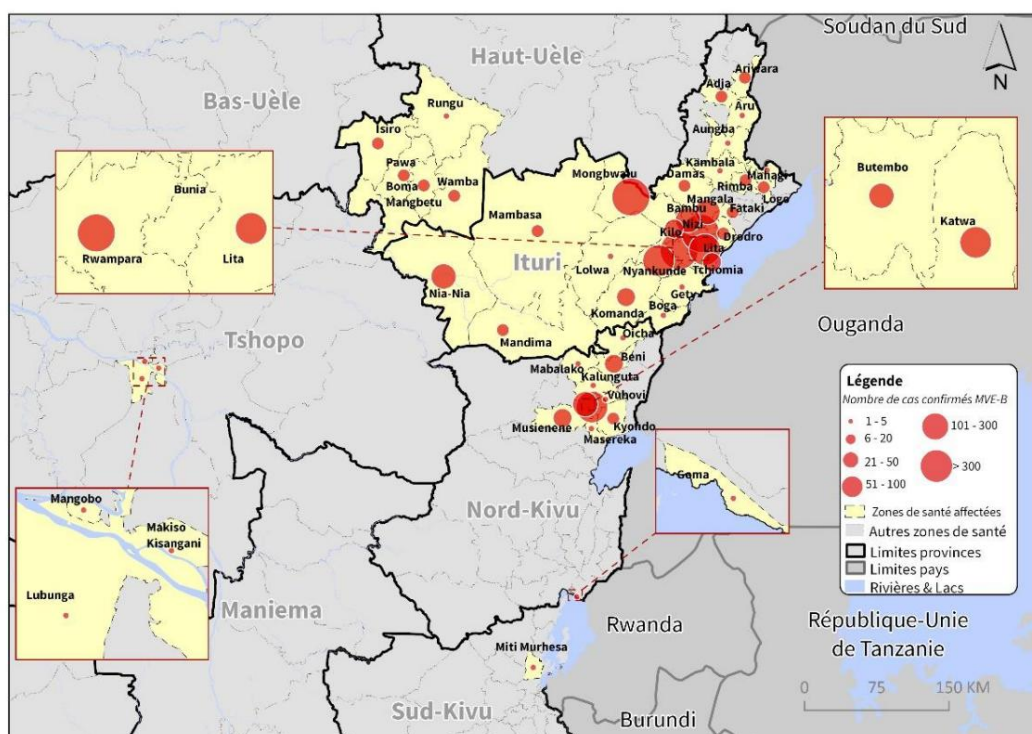
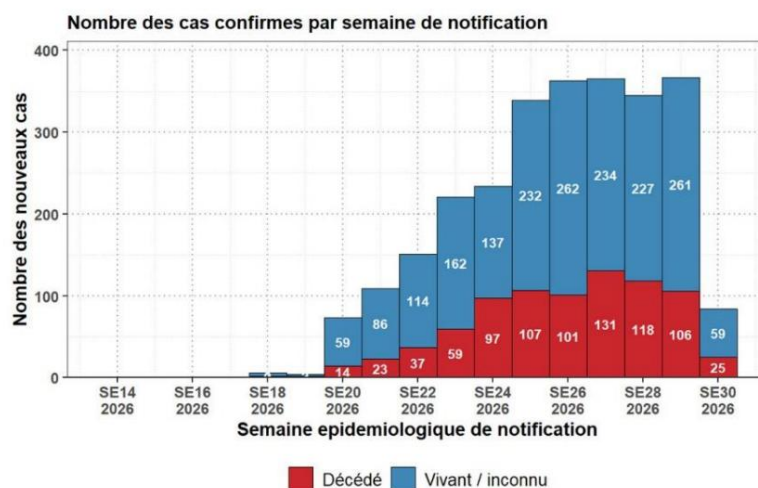


FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

The recent emergence of the new Rungu health zone in Haut-Uélé confirms a progressive expansion of affected areas, highlighting the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S25 to S30, with fluctuations related to notification delays and the gradual consolidation of data.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses : cas confirmés avec statut vital et date de notification renseignés, n = 2 661.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

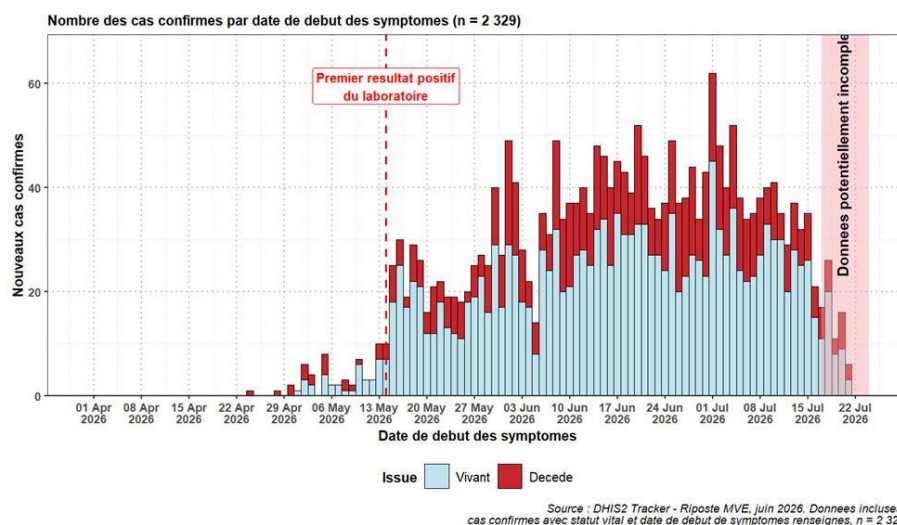


FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

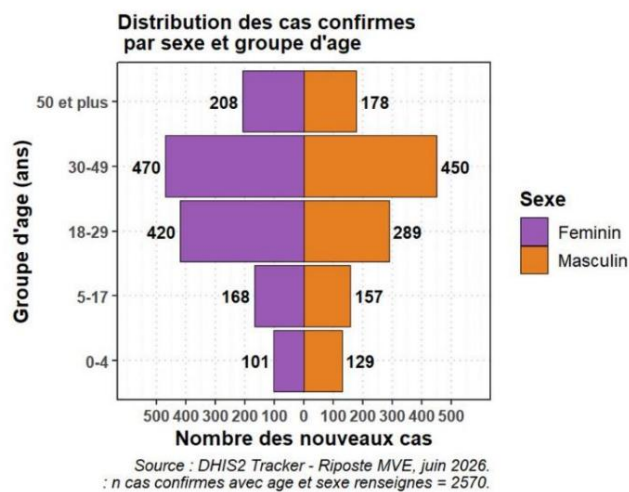
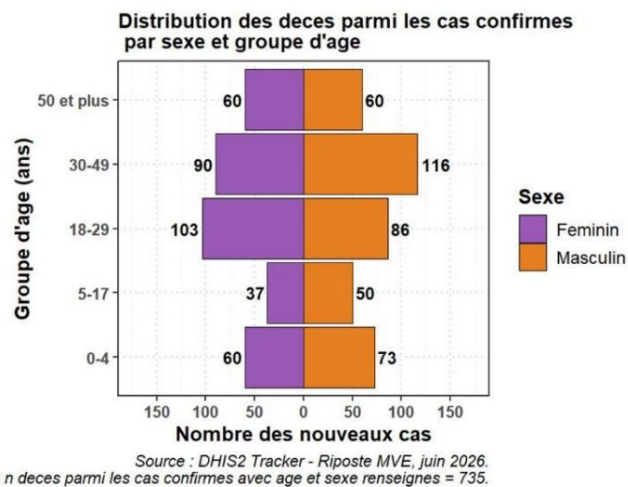


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu	Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	18	24	ND	6	ND	48
New alerts received Total	532	924	ND	27	ND	1,483
alerts for today Alerts	550	948	ND	33	ND	1,531
investigated Investigation	296	917	ND	9	ND	1,222
rate (24 h)	53.8%	96.7%	ND	27.3%	ND	79.8%
Validated alerts — alive Validated	137	64	ND	8	ND	209
alerts — deceased (comm.)	62	43	ND	1	ND	106
Total suspected cases for today	199	107	ND	9	ND	315

In total, 1,531 alerts (including 1,483 new ones) were received, with 1,222 investigated within 24 hours (79.8%) and 315 validated (209 active cases and 106 community deaths). This improvement reflects a reduction in investigation delays and the consolidation of data from the affected provinces. The improved investigation rate is linked to the reduction in investigation delays and the strengthening of surveillance teams in the affected provinces.

3.2 Laboratory: tests and positivity

• **Ituri** : 155 samples documented in the collection network, of which 51 were positive (33 living and 18 deceased), i.e., a positivity rate apparent of 32.9%.

• **North Kivu** : 120 samples received and analyzed, of which 15 were positive (7 living and 8 dead), representing a positivity rate of 12.5%.

• **Tshopo** : 3 samples received and analyzed. All of these samples came back negative after analysis.

- *Installation and launch of activities of the diagnostic laboratory in Kasenyi in the Tchomia health zone in Ituri*
As part of the DRC-Uganda cross-border coordination, with support from WHO and US CDC, the WHO donated two RadiOne machines to the Kasenyi laboratory.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts being followed	Viewed contacts	tracking rate
Ituri	11,332	8,170	72.1%
North Kivu	4001	3,156	78.9%
Tshopo	25	25	100.0%
Haut-Uélé	ND	ND	ND
South Kivu (end of monitoring)	0	0	N / A
Total	15,358	11,351	73.9%

Contact tracing remains insufficient at the national level (73.9%), below the operational threshold of 95%. Ituri only managed to follow up on 8,170 contacts out of 11,332 (72.1%). North Kivu achieved a performance of 3,156 contacts seen out of 4,001. (78.9%). Tshopo maintains a high performance with a 100.0% follow-up rate. Follow-up is closed in South Kivu.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 5 alerts notified

Five (5) alerts were notified to the PoE/PoC in Ituri. All of these 5 active alerts were validated and all referred to the CTE/CT.

North Kivu — 5 alerts notified

Five (5) alerts were notified to the PoCs, including 3 live and 2 dead bodies; 1 live alert was validated and referred to the CT of HGR Oicha; the two bodies were referred for swab.

Tshopo — 0 alerts notified

No alerts were notified to the various PoE/PoC points. It should be noted that 2,415 travelers passed through the PoE/PoC points, of which 2,405 were screened, 2,379 washed their hands, and 2,415 received awareness training as of July 22, 2026.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (16/16)	100% (76/76)
Number of people who went through PoE/PoC	104656	63813	168469
testing, % of travelers	95%	97.7%	95.3%
screened, % of travelers who washed	94%	97.7%	95.2%
their hands, % of travelers made aware of the issue	96.1%	97.7%	96.5%
Number of alerts notified	5	5	10
Number of validated live alerts	5	5	10
Number of bodies recovered today	0	2	2
Number of problematic contacts intercepted today	0	0	0
Percentage of validated live alerts referred to the CT/CTE	100%	100%	66.7% (2/3)
% of lifeless bodies swabed	100.0% (1/1)	100%	100.0% (1/1)
Number of positive results of referred suspected cases	ND	0	0

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI)

Indicator		Ituri	North Kivu	Haut-Uele	Tshopo	Together
Number of beds		709	141	ND	10	860
Patients in bed (Day -1)		551	170	ND	3	724
Total admissions (24 hours)		107	61	ND	1	169
Total admissions		658	231	ND	4	893
Outings	Deceased	23	1	ND	0	24
	No case	42	36	ND	0	78
	Healed	20	1	ND	0	21
	Escapees	3	1	ND	0	4
	Transferred to the HGR	0	0	ND	0	0
Total outflows (24 h)		88	39	ND	0	127
Patients in isolation (end of day),		570	192	ND	4	766
including confirmed cases		258	20	ND	0	278

including suspects	312	172	ND	4	488
Occupancy rate	80.4%	136.2%	ND	40.0%	89.1%

In total, 766 patients were in isolation, including 278 confirmed cases and 488 suspected cases, for an overall occupancy rate of 89.1 % (766 patients for 860 beds), with 893 admissions and 127 discharges (21 recovered, 24 deceased, 78 non-cases, and 4 escaped). Ituri bears the brunt of the burden with 570 patients in isolation (258 confirmed, 312 suspected) out of 709 beds, representing an 80.4% occupancy rate. In North Kivu, 192 patients are in isolation (20 confirmed, 172 suspected) for an occupancy rate of 135.5% (141 beds). Tshopo province has a 40.0% occupancy rate. Haut-Uele did not make data available at the time of compilation.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	16 (66.7%)	5 (100%)
Confirmed cases are being monitored.	150 (96.8%)	13 (100%)
New suspected cases	39 (46.4%)	6 (100%)
Suspected cases are being monitored	103 (57.2%)	16 (100%)
Laboratory results announced (including positive ones)	43	12
Children separated — nursery / community	16	5
CTE support staff	0	0
Frontline Personnel (PPL)	224	0
Members of households and communities	36	6
EDS supported (SMSPS)	6	6

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD

Health Zone	Confirmed cases	Cured. In hospital.		Death
Bunia	35	18	3	14
Rwampara	32	24	1	7
Mongbwalu	16	6	2	8
Nyankunde	14	11	1	2
Nia-Nia	10	3	7	0
Lita	4	1	2	1
Nizi	3	0	0	3
Mangala	3	1	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Logo	2	0	0	0
Total — case fatality rate 29.3%	123	67	20	36

PPL = healthcare personnel and frontline providers.

7 Pillar-Based Response Activities

COORDINATION Arrival of Her Excellency the Prime Minister in the provinces of Tshopo and Ituri; High-level strategic meeting with Her Excellency the Prime Minister; Donation of 10 vehicles to the province of Ituri; Visit to the laboratory provincial of Tshopo by Her Excellency the Prime Minister and of the site planned for the construction of the CTE/EOC by the Governor of Tshopo; Continuation of coordination meetings and supervision of field activities; coordination of the pillars and support for the installation of local laboratories.	PCI / WASH ÿ Ituri: 18 rings opened, 35 ESS monitored, 10 assessments, 9 ESS and 8 means of transport decontaminated, 182 PPL trained, 44/107 households decontaminated within 48 hours. ÿ North Kivu : Decontamination of 4 health facilities (HCFs) for case stays, including 1 in Beni, 2 in Butembo and 1 in Katwa; Provision of an incomplete IPC kit in 1 HCF in Butembo (CH MALENDE); IPC scorecard evaluation in 8 HCFs, including 1 in Beni, 5 in Butembo, and 2 in Katwa; Monitoring and support of 59 HCFs, including: 18 in Beni, 11 in Butembo, 2 in Katwa, 2 in Kyondo and 26 in Oïcha; Briefing of 209 HCF providers during formative supervision, including: 67 in Beni, 38 in Butembo, 9 in Katwa, 9 in Kyondo and 86 in Oïcha; Community deaths benefiting from EDS: 8, including 6 in Beni, 1 in Butembo and 1 in Oïcha; Community deaths that were subject to SWAB: 11, including 6 in Beni, 1 in Butembo, 2 in Kondo, 1 in Mabalako and 1 in Oïcha. ÿ Tshopo : PCI Scorecard assessments carried out at CS Kwetu (16.1%) and CS Jamaa (37.1%). Decontamination of 10 households (AS St André) and 2 social service establishments (AS St André & AS Libota). Installation of the triage service at the Tshopo General Referral Hospital.
Dignified and secure burials (DSM) ÿ Ituri: 89 death alerts, 71 EDS carried out, 49 swabs; EDS proportion 80%. Community resistance in Nyankunde and Nizi; EDS training in Tchomia. ÿ North Kivu: 38 Swabs carried out out of 40 death alerts and 40 EDS completed. ÿ Tshopo: continuation of the required Coordination for the management of community deaths and the safe transport of bodies.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿ Ituri: Continued supervision of health zone.
COMMUNICATION (RCCE / CREC) ÿ Ituri: 1,053 people were reached by awareness messages delivered by RECO 54 in households in 1 out of 4 Health Zones; communities are beginning to apply handwashing without problems; 70 people were reached by awareness themes during educational talks held by community actors in health zones; 62 people, including village chiefs and leaders of women's associations, were engaged in response actions following community dialogue sessions held in the Health Zones;	HOLISTIC CARE (HCC) ÿ Ituri: 20 new recoveries, 570 patients in isolation, 23 deaths among those discharged; 1024 hot meals were served to patients, including 121 to suspected cases, 122 to confirmed cases, 723 to PPLs and 58 to accompanying persons. ÿ North Kivu: continued monitoring of care in CTE/CT. ÿ Tshopo: 0 new admissions today. 4 suspected cases currently hospitalized at the CTE HC Kisangani. Second PCR test performed on 2 suspects (results in

2,755 people were reached during mass awareness sessions held by community actors in the Health Zone (AUNGBA, BUNIA, MONGBWALU): including 38 women on Ebola Virus Disease (EVD) with financial support from MEDAIR; 34 religious leaders, including 14 women, were briefed on EVD prevention in their respective communities, and the procedures to follow through different pillars; 35 Community Health Workers (CHWs), heads of blocks/religious leaders and association leaders at the ISP site were trained on EVD, community-based surveillance, community dialogue, and contact tracing at the site, including 14 women, with support from CARITAS (RWAMPARA, NIZI). ÿ North Kivu: Briefing of 38 RECOs, cell leaders and neighborhood chiefs on the operationalization of SBC activities, in AS KYAGHALA in the ZS of Katwa; Mass communication to 123 people on the importance of contact tracing and the role of food kits distributed with the support of PAM. ÿ Tshopo: 60 VADs (Virtual Access Debriefings) conducted (360 people reached). Mass awareness campaign targeting 15,868 people in 5 health zones. Training provided to 35 CREC members, 22 journalists, 712 RECOs (Research and Education Coordinators), 217 community leaders, and 414 motorcycle taxi drivers.	waiting). 1 operational ambulance based at Makiso General Hospital.
RESEARCH, INNOVATION & VACCINATION ÿ Ituri: national and provincial meeting with research partners; deployment of CAP investigators to Rwampara, Bunia, Bambu Mines, Kilo, Lita, Gethy and Komanda; anthropological publications in progress	LOGISTICS ÿ Ituri: Ensure the mobility of the different pillars. ÿ North Kivu: Delivery of 40 bottles of 200ml of Infacare milk to the nursery of the Beni General Referral Hospital in the Beni Health Zone; Deployment of service providers, section heads and pillars to Butembo to participate in the SGI meeting; Deployment of the mitigation & security section teams in the Beni health zone / Mukulya checkpoint; Carry out the daily activities of the section. ÿ Tshopo: Distribution of investigation/alert forms in the 4 urban security zones. Provision of fuel. Identification of the 100m × 100m plot of land for the new CTE.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ÿ Ituri: Continuation of activities on the ground.	

8 Main challenges, impacts and actions required

Challenge No.	Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of governor military of Ituri)	Interruption of response activities, including dignified and safe burials Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT: occupancy of 136.2% in North Kivu (bed capacity: 141) and 80.4% in Ituri (570 patients for 709 beds);	Admission delays, increased risk of nosocomial transmission Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity
4	Low contact tracing performance in Ituri (72.1%); North-Kivu 78.9%	Uninterrupted transmission chains Train, deploy and motivate community liaison officers (RECOs); strengthen supervision; investigate the observed drop in Ituri
5	Insufficient number of ambulances and pre-positioning at PoC	Delays in transfer and isolation Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Weakening of overall care Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (175/1170, i.e. 15%)	pupil infections Nosocomial risks (123 cases of PPL including 36 deaths) Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of laboratories proximity (Butembo, Mambasa and Logo)	Long confirmation delays, transmission not detected Accelerate the deployment of decentralized diagnostic capabilities
10	Financial shortcomings of the resources	Limitation of the implementation of the pillars Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



Official handover of 10 vehicles by Her Excellency the Prime Minister to the IM to strengthen the response against Ebola in the Ituri province (UG PDSS- Prime Minister's Office)



Visit of Her Excellency the Prime Minister to the provincial laboratory of Kisangani in Tshopo



Installation and launch of activities of the diagnostic laboratory in Kasenyi in the Tchomia health zone in Ituri

**POUR TOUTE
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Avec l'appui technique de l'Organisation Mondiale de la Santé (OMS),
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